Ovarian Mass Ultrasound: Guideline Review & Scanning and Reporting Protocol

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Purpose and scope

This protocol standardises how adnexal masses are scanned, described, risk-stratified and reported, so that every report carries an IOTA-based description, a validated risk estimate and a clear management recommendation.

- **Applies to:** all transvaginal and transabdominal gynaecological ultrasound where an adnexal (ovarian, para-ovarian or tubal) lesion is identified, including incidental findings and follow-up scans.
- **Users:** sonographers, sonologists, O&G and MFM specialists, trainees and Western Sydney University student sonographers on placement.
- **Excludes:** acute presentations where O-RADS/IOTA risk models do not apply (suspected torsion, ectopic pregnancy, tubo-ovarian abscess/PID) — these are reported descriptively with urgent clinical communication; adnexal masses in pregnancy are scanned with the same lexicon but risk models are interpreted with caution.
- **Approach adopted:** IOTA terminology for description, a two-step IOTA strategy (Simple Rules / Simple Rules risk, then ADNEX for inconclusive or complex lesions), with an O-RADS US v2022 category added to every report for referrers and radiology interoperability.

Current practice and risk-stratification systems

International practice has moved from CA125-weighted scores (RMI) to ultrasound-based IOTA models, with the IOTA two-step strategy (modified benign descriptors, then ADNEX) now the IOTA Steering Committee's recommended approach and O-RADS US v2022 the common language for risk category and management.

Transvaginal ultrasound by an experienced examiner is the first-line test everywhere. Subjective assessment by a Level III (expert) examiner performs best; validated models are recommended when expert review is not available, and to make reports reproducible.

System	What it does	Output	Strengths	Limitations
IOTA terms & definitions (2000; updated 2026)	Standard lexicon and measurement rules	Description only	Foundation for every IOTA model and O-RADS	Must be applied exactly for models to work
IOTA modified benign descriptors (BD1–4)	Instant benign diagnosis for unilocular lesions <10 cm	Benign / not applicable	Apply to ~37% of masses; 99.3% of those benign in IOTA5	Only covers simple-pattern lesions
IOTA Simple Rules (5 B + 5 M features)	Benign / malignant / inconclusive classification	3 groups	No computer needed; applies to ~80% of masses	~20% inconclusive; no risk estimate
IOTA Simple Rules risk	Logistic model on the 10 SR features + centre type	% risk	Gives a risk for every mass	Needs calculator
IOTA ADNEX	Multiclass risk model (9 variables, CA125 optional)	% benign, borderline, stage I, stage II–IV, metastatic	Best-performing model with SR risk; avoids subjective variables; works for Level II examiners	Needs app/machine software; CA125 optional
ACR O-RADS US v2022	Lexicon-based risk category with management	O-RADS 0–5	Management attached to each category; maps to ADNEX risk bands	Descriptor route less specific than ADNEX in O-RADS 3–4
RMI-I	U score × menopausal status × CA125	Score (>200 = refer)	Simple; still used in Australian GP and UK pathways	CA125-dependent; poor for early-stage and borderline disease, especially premenopausal

System	What it does	Output	Strengths	Limitations
GI-RADS	5-category descriptive system	GI-RADS 1–5	Simple	Less validated; not adopted by major guidelines

The 2021 ESGO/ISUOG/IOTA/ESGE consensus found the IOTA ADNEX and Simple Rules risk models outperform the RMI (Grade A). A randomised trial cited there showed IOTA Simple Rules triage reduced surgery compared with the RMI-based RCOG protocol without delaying cancer diagnoses.

National and RANZCOG recommendations

Australia has no single national adnexal-mass imaging guideline: ASUM sets scanning standards and asks for IOTA description, while Cancer Australia and RACGP referral guidance still use the RMI (>200 = gynaecological oncology referral).

- **RANZCOG.** No current standalone RANZCOG clinical statement on ultrasound characterisation of adnexal masses was found. RANZCOG's *O&G Magazine* "The Ovary" issue (Autumn 2020) describes the IOTA approach — classic lesions, Simple Rules and O-RADS — and gyn-oncology referral of suspicious masses (O&G Magazine). RANZCOG trainees and Fellows are therefore working to international (IOTA/ESGO/RCOG) standards. *Open question: confirm with RANZCOG Women's Health Committee whether a statement is in development.*
- **ASUM.** The ASUM gynaecological scan guideline lists ovaries and adnexa as mandatory structures and directs that cysts and solid masses be described according to IOTA; a 2026 edition, *Guidelines on the Performance of a Gynaecological Ultrasound Examination*, is listed on ASUM's standards page (ASUM standards; 2020 draft).
- **Cancer Australia.** For symptoms persisting over a month, GPs should request a transvaginal ultrasound plus serum CA125, calculate the RMI, and refer to a gynaecological oncologist when it exceeds 200; CA125 alone should not be used (GP card, 2019). The *Optimal Care Pathway for women with ovarian cancer* sets the expectation that confirmed or suspected cases are referred to a gynaecological oncologist (OCP quick reference).
- **RACGP.** The AFP review advises urgent gyn-oncology assessment for RMI >200, referral for any IOTA M-rule feature, and discussion with gyn-oncology when CA125 exceeds about 250 U/mL (AFP 2015).

Implication for this protocol: referrers in NSW will expect an RMI-compatible report. Every report should therefore state the U-score features (multilocular, solid, bilateral, ascites, metastases) alongside the IOTA/O-RADS result, so a GP can calculate RMI if they

wish, while the clinical recommendation follows IOTA/O-RADS.

International guideline recommendations

The European societies recommend expert ultrasound or IOTA ADNEX as the primary triage tool; the ACR recommends O-RADS; the UK and US national guidelines still reference RMI and CA125 but have incorporated IOTA Simple Rules.

ESGO/ISUOG/IOTA/ESGE Consensus (2021) — 18 statements (full text):

1. Subjective assessment by a Level III examiner is the best discriminator (Grade A).
2. Where no expert is available, ultrasound-based models should be used (B).
3. IOTA SR risk or ADNEX is preferred over CA125, HE4 or ROMA (B).
4. ADNEX and SR risk outperform RMI and other scoring systems (A).
5. ADNEX helps separate benign, borderline, stage I, stage II–IV and secondary metastatic tumours (C).
6. An ADNEX metastatic-tumour risk ≥10% should trigger a search for a primary elsewhere (D).
7. CA125 is the best single biomarker but adds little to ultrasound models; CEA and CA19-9 help identify secondary tumours.
8. MRI (with DCE and DWI) is a second-line test after ultrasound for indeterminate masses (B); imaging alone cannot predict peritoneal resectability.
9. Pathway: physiological finding (O-RADS 1) → discharge or rescan at 6 weeks; <1% (O-RADS 2) → conservative follow-up at 3–6–12 months if asymptomatic; 1–<10% (O-RADS 3) → general gynaecology; ≥10% (O-RADS 4–5) → gynaecological oncology centre.

IOTA updated terms and definitions (April 2026) — the IOTA Steering Committee now recommends the benign descriptors, ADNEX and the two-step strategy, and the 2026 paper updates definitions users found ambiguous. It maps ADNEX risk directly to O-RADS: BDs or <1% = O-RADS 2; 1–<10% = O-RADS 3; 10–<50% = O-RADS 4; ≥50% = O-RADS 5 (UOG 2026; 67:702–724). In the IOTA5 validation, the two-step strategy had an AUC of 0.94 (Landolfo 2023).

ACR O-RADS US v2022 — six categories (0 incomplete, 1 normal, 2 almost certainly benign <1%, 3 low risk 1–<10%, 4 intermediate 10–<50%, 5 high ≥50%), with separate pre- and postmenopausal management. v2022 added the *bilocular* cyst and *acoustic shadowing* in smooth solid lesions as benign features, allows short-interval US follow-up for O-RADS 3 cystic lesions, keeps gyn-oncology involvement for O-RADS 4 and referral

for O-RADS 5, and states that each lesion gets its own score with management driven by the highest (ACR update; RSNA News 2025). It does not apply to acute settings such as torsion, ectopic pregnancy or PID (Radiopaedia).

RCOG — Green-top 62 (premenopausal, 2011) recommends IOTA Simple Rules, referral of any M-rule mass to gyn-oncology, specialist ultrasound for inconclusive masses, and gyn-oncology discussion when CA125 >200 U/mL (summary). Green-top 34 (postmenopausal, 2016) uses RMI-I with CA125 for triage. The ROCkeTS study now argues for IOTA ADNEX at a 10% threshold as the new standard in the UK (BMUS 2025).

NICE — CA125 then abdominopelvic ultrasound for women ⩾40 with persistent symptoms; AFP and β-hCG as well as CA125 for women under 40; CT for staging when ultrasound, CA125 and clinical status suggest cancer; MRI not routine (NICE summary).

ACOG Practice Bulletin 174 (2016) — gyn-oncology referral for postmenopausal women with elevated CA125, or for any woman with suspicious ultrasound findings, ascites, metastatic disease or high IOTA/biomarker risk scores (ObG Project summary).

SOGC Guidelines 403/404 (2020) — initial ultrasound using pattern recognition or IOTA models, with referral for features suspicious of malignancy (reference list).

Guideline comparison

All guidelines agree on transvaginal ultrasound first, standardised (IOTA) description and gyn-oncology referral for suspicious masses; they differ mainly on whether CA125/RMI or an ultrasound model drives triage.

Body (year)	Primary triage tool	Role of CA125	Lexicon	Referral trigger
IOTA (2026 update)	Modified BDs → ADNEX	Optional ADNEX input	IOTA	ADNEX ⩾10% (O-RADS 4–5)
ESGO/ISUOG/IOTA/ESGE (2021)	Expert subjective assessment or ADNEX / SR risk	Adjunct; subtype discrimination	IOTA + O-RADS categories	Risk ⩾10%
ACR O-RADS US (v2022)	O-RADS descriptors or ADNEX	Not required	O-RADS (IOTA-based)	O-RADS 4 (gyn-onc consult) or 5 (referral)

Body (year)	Primary triage tool	Role of CA125	Lexicon	Referral trigger
RCOG GTG 62 / 34 (2011 / 2016)	Simple Rules (pre); RMI-I (post)	RMI; >200 U/mL premenopausal prompts discussion	IOTA	Any M-rule; RMI >200
NICE CG122 / NG12	CA125 → ultrasound; RMI	Central	Not specified	Suspicious ultrasound + CA125
ACOG PB 174 (2016)	Ultrasound ± biomarker panels	Central in postmenopausal	Not specified	Suspicious US, ascites, raised CA125
Cancer Australia / RACGP	TVUS + CA125 → RMI	Central	Not specified	RMI >200; any M-rule
ASUM	Scanning standard (not a triage tool)	—	IOTA	—
RANZCOG	No formal statement found	—	IOTA (educational material)	Gyn-oncology for suspected malignancy

Where they agree: TVUS (with transabdominal for large masses) is first-line; describe with IOTA terms; colour Doppler is part of the assessment; MRI is second-line for indeterminate masses; suspected malignancy goes to a gynaecological oncologist.

Where they differ: RMI and CA125 still drive Australian GP and UK postmenopausal pathways, while ESGO/IOTA/ACR place ultrasound models first. This protocol reports both, with IOTA/O-RADS determining the recommendation.

Scanning protocol

Every adnexal lesion is examined transvaginally (plus transabdominally if large), described with IOTA terms, measured in three orthogonal planes, scored with colour Doppler, and then run through the two-step strategy before the patient leaves.

A. Before the scan

1. **Clinical data (mandatory for risk models):** age, menopausal status (postmenopausal = ≥12 months amenorrhoea, or age ≥50 after hysterectomy), LMP/cycle day, hormonal therapy, pregnancy status, symptoms (pain, bloating, early satiety, bleeding), personal/family history of ovarian, breast, bowel or endometrial cancer, known BRCA/Lynch status, prior adnexal surgery, endometriosis.
2. **Bloods if available:** CA125 (date and value); CEA/CA19-9, AFP, β-hCG, LDH as clinically indicated. Absence of CA125 does not delay the scan — ADNEX can run without it.
3. **Consent and chaperone** for the transvaginal examination per ASUM; offer transabdominal/transrectal alternatives where TV is declined or unsuitable.
4. **Bladder:** empty for TVUS; partially full for an initial transabdominal survey of large masses.

B. Equipment and settings

- High-frequency endovaginal probe (typically 5–9 MHz) and curvilinear abdominal probe (2–5 MHz).
- Grey-scale optimised: focus at the lesion, depth so the lesion fills about two-thirds of the screen, harmonics on.
- Colour/power Doppler: low pulse repetition frequency (about 0.3–0.6 kHz), low wall filter, colour gain raised until noise appears then reduced just below it. Use the same settings at follow-up.
- IOTA ADNEX/Simple Rules software on the machine, or the IOTA app/online calculator.

C. Systematic survey

1. Uterus (size, myometrium, endometrial thickness and pattern — a thick endometrium with a solid ovarian mass raises granulosa-cell tumour or synchronous endometrial cancer).
2. Right and left ovary: identify each; measure in three planes; record antral follicles if normal.
3. Locate every lesion: ovarian, para-ovarian, tubal, or uncertain. Use the sliding/split sign and pressure with the probe to separate ovary from lesion. Consider mimics (pedunculated fibroid, hydrosalpinx, peritoneal inclusion cyst, bowel, pelvic kidney).
4. Tenderness and mobility of each ovary/lesion ("soft markers"); sliding sign for pouch of Douglas obliteration.
5. Free fluid: pouch of Douglas only (record depth) vs **ascites** (IOTA: fluid outside the pouch of Douglas).

6. Transabdominal survey of upper abdomen when a lesion is suspicious (O-RADS ≥4 or any M feature): ascites in Morison's pouch, omental thickening/cake, peritoneal nodules, liver surface, hydronephrosis, pleural effusion.

D. Lesion characterisation (IOTA terms)

For each lesion, record:

1. **Size:** three orthogonal diameters of the whole lesion (largest diameter first, in the plane where it is longest; rotate 90° for the third).
2. **Tumour type:** unilocular · unilocular-solid · multilocular · multilocular-solid · solid (solid tissue ≥80% of the lesion on the 2D section). Count locules; record whether >10.
3. **Cyst wall and septa:** smooth or irregular (irregularity <3 mm is not a papillary projection).
4. **Cyst content echogenicity:** anechoic · low-level · ground-glass · haemorrhagic · mixed.
5. **Solid components:** presence; maximum diameter of the largest solid component in three planes (papillary projections are included as solid components; the largest papillation is measured in three planes).
6. **Papillary projections:** solid tissue projecting into the cyst cavity with height ≥3 mm. Count: 0, 1, 2, 3, >3. Note surface (smooth/irregular) and flow within them.
7. **Solid lesion surface:** regular or irregular (O-RADS: smooth vs irregular contour).
8. **Acoustic shadowing:** present/absent (a strong benign feature — dermoid, fibroma, Brenner).
9. **Colour score** of the whole lesion: 1 no flow · 2 minimal · 3 moderate · 4 abundant. Record flow in solid components separately.
10. **Bilateral:** yes/no; if bilateral, characterise and score each lesion separately.
11. **Pattern recognition:** suggested diagnosis (e.g. endometrioma, dermoid, haemorrhagic cyst, cystadenofibroma, hydrosalpinx) and degree of confidence.

E. Risk assessment — apply in order

1. **Normal / physiological?** Follicle or corpus luteum ≤3 cm in a premenopausal woman → O-RADS 1; stop.
2. **Step 1 — IOTA modified benign descriptors** (all require largest diameter <10 cm):
   - BD1: unilocular, ground-glass, premenopausal (endometrioma)
   - BD2: unilocular, mixed echogenicity with acoustic shadows, premenopausal (dermoid)
   - BD3: unilocular, anechoic, regular walls
   - BD4: any other unilocular cyst with regular walls

If one applies → almost certainly benign (O-RADS 2).

3. **Step 2 — IOTA ADNEX** when no BD applies. Enter: age, CA125 if available, centre type (record as "non-oncology centre" unless the patient is scanned in a tertiary gyn-oncology unit), maximum lesion diameter, maximum diameter of largest solid component, >10 locules, number of papillary projections, acoustic shadows, ascites. Record total risk of malignancy and the five subclass probabilities.
4. **Cross-check with IOTA Simple Rules** (B1–B5, M1–M5) and record the result — this keeps the report RCOG/RACGP-compatible and flags any M feature.
5. **Assign O-RADS US v2022 category** from the ADNEX risk band (IOTA 2026 mapping) or O-RADS descriptors, whichever is higher. Where pattern recognition by a Level III examiner disagrees with the model, state both and explain.
6. **Escalation in clinic:** an M feature, ADNEX ≥10%, or ADNEX metastatic risk ≥10% → same-day review by the reporting specialist before the patient leaves.

F. Images to store (minimum)

- Each ovary in two planes with measurements.
- Lesion: three orthogonal measurements; the largest solid component measured; each papillary projection; any shadowing; the most complex section.
- Colour Doppler image at the most vascular area (with colour score).
- Cine clip sweeping through the whole lesion (recommended for MDT and audit review).
- Pouch of Douglas fluid; upper-abdomen images if suspicious.

G. Special situations

- **Pregnancy:** same lexicon; ADNEX is not yet validated in pregnancy (p-IOTA study ongoing) — interpret with caution; decidualised endometriomas can mimic malignancy.
- **Suspected torsion, ectopic, TOA/PID:** report descriptively, do not assign O-RADS; urgent verbal communication.
- **High-risk patients (BRCA/Lynch):** apply the protocol, but lower the threshold for specialist referral; O-RADS management may be modified by clinical risk.
- **Postmenopausal ovaries not seen:** document; transabdominal attempt; state "ovaries not visualised" rather than "normal".

Reporting protocol

Every report carries the same structure: IOTA description, the two-step result (BD or ADNEX risk), Simple Rules features, an O-RADS v2022 category, a suggested diagnosis and one explicit management recommendation with a timeframe.

Structured report template

```
ADNEXAL ULTRASOUND REPORT
Clinical: [age] | [pre/postmenopausal] | LMP/day [ ] | Indication [ ]
Risk factors: [family history / BRCA / Lynch / none] | CA125: [value, date /
not available]
Technique: Transvaginal ± transabdominal, grey-scale and colour Doppler.
Image quality: [good / limited – reason]

UTERUS: [size, endometrium thickness/appearance, other findings]
RIGHT OVARY: [dimensions] [normal / contains lesion R1]
LEFT OVARY: [dimensions] [normal / contains lesion L1]

LESION [R1]:
  Origin: [ovarian / para-ovarian / tubal / uncertain]
  Size: [a × b × c mm]
  IOTA type: [unilocular / unilocular-solid / multilocular / multilocular-
solid / solid]
  Locules: [n; >10 Y/N]   Walls/septa: [smooth / irregular]
  Content: [anechoic / low-level / ground-glass / haemorrhagic / mixed]
  Solid component: [absent / present – largest a × b × c mm]
  Papillary projections: [0 / 1 / 2 / 3 / >3]; flow [Y/N]
  Solid surface: [regular / irregular / n.a.]   Acoustic shadowing: [Y/N]
  Colour score: [1–4]

FREE FLUID: [none / POD only – depth mm / ascites]
UPPER ABDOMEN (if assessed): [peritoneal nodules / omental cake /
hydronephrosis / none]
TENDERNESS / MOBILITY: [ ]

RISK ASSESSMENT:
  IOTA modified benign descriptor: [BD1–4 applies / not applicable]
  IOTA ADNEX [with / without CA125]: risk of malignancy [ %]
     benign [ %] | borderline [ %] | stage I [ %] | stage II–IV [ %] |
metastatic [ %]
  IOTA Simple Rules: B features [ ] | M features [ ] → [benign / malignant /
inconclusive]
  RMI U-score features: [multilocular / solid / bilateral / ascites /
metastases]
  O-RADS US v2022: [0–5] – [risk category]
```

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CONCLUSION:
  1. [Most likely diagnosis] [right/left] [size], O-RADS [ ], ADNEX [ %].
  2. Recommendation: [explicit action + timeframe – see management table]
  3. [Comparison with prior study, interval change]
Reported by: [name, qualification]   Sonographer: [name]   Date/time of
verbal communication if applicable: [ ]
```

Management recommendations by category

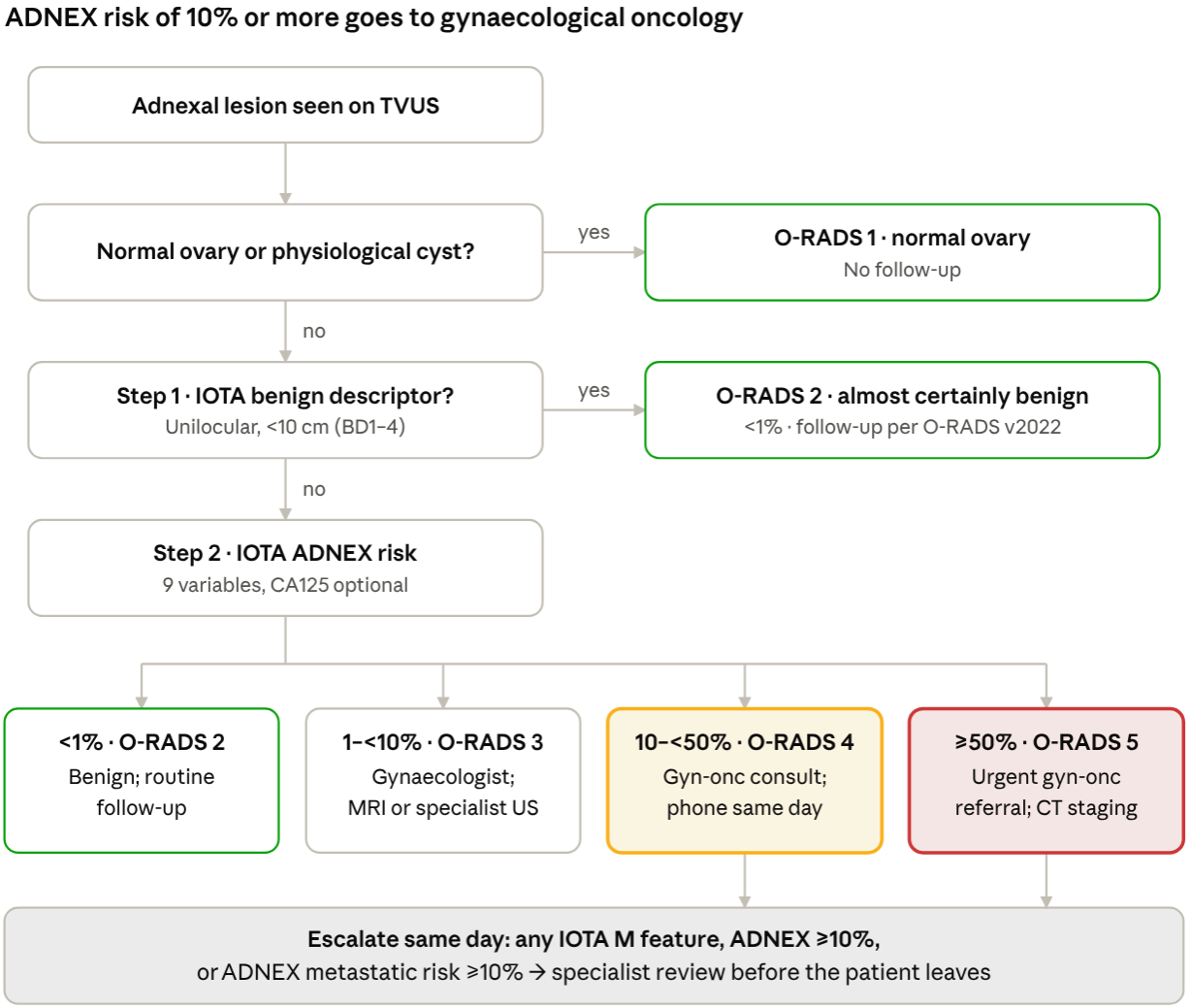
O-RADS (risk)	Typical findings	Recommendation in report	Communication
0 – Incomplete	Lesion not fully assessable (bowel gas, size, poor access)	Repeat TVUS/TAUS or MRI; state the reason	Routine
1 – Normal	Follicle or corpus luteum ⩽3 cm (premenopausal)	No follow-up	Routine
2 – Almost certainly benign (<1%)	Modified BD applies; simple, bilocular smooth, or classic benign lesion <10 cm	Asymptomatic: per O-RADS v2022 classic-lesion table (e.g. none for small simple cysts; rescan in 8–12 weeks for larger haemorrhagic/simple premenopausal cysts; 12-monthly for dermoid/endometrioma not excised)	Routine
3 – Low risk (1–<10%)	ADNEX 1–<10%; smooth multilocular, smooth solid with shadowing, unilocular with small solid component	Gynaecologist review; options: specialist US (if not already Level III), MRI with O-RADS MRI score, or short-interval US for cystic lesions; surgery in a general gynaecology unit if indicated	Report within 24 h to referrer

O-RADS (risk)	Typical findings	Recommendation in report	Communication
4 – Intermediate (10–<50%)	ADNEX 10–<50%; multilocular-solid, unilocular-solid, solid with smooth contour and colour score 2–3	Gynaecologist with gyn-oncology consultation, or direct gyn-oncology referral; MRI for characterisation; CA125 ± CEA/CA19-9 if not done	Phone referrer same day
5 – High (≥50%)	ADNEX ≥50%; ≥4 papillary projections, irregular solid, ascites/peritoneal nodules	Urgent gyn-oncology referral (e.g. Westmead Hospital gynaecological oncology); CT chest/abdomen/pelvis per gyn-onc protocol	Phone referrer same day; document time and recipient

Additional triggers regardless of category: any IOTA M feature; ADNEX metastatic risk ≥10% (recommend search for a GI, breast or other primary, plus CEA and CA19-9); RMI >200 when CA125 is known.

Follow-up scans: compare with the prior study, state interval change in size and morphology, re-run the two-step strategy, and stop surveillance when the lesion resolves or after stability per O-RADS v2022 (management to be verified against the current ACR tables before go-live).

Triage pathway



IOTA two-step triage mapped to O-RADS US v2022 actions

Physiological findings and benign descriptors exit early; everything else gets an ADNEX risk, and the O-RADS band sets who manages the patient and how fast the referrer is called.

Quality assurance, training and audit

Diagnostic accuracy depends on examiner expertise, so the protocol is backed by IOTA certification, double-reporting of higher-risk lesions and an annual audit against histology and follow-up outcomes.

- **Competency:** all sonographers and sonologists complete the IOTA online certification (or equivalent) before independent reporting; refresh when the IOTA definitions change (latest April 2026).

- **Expertise levels:** ESGO consensus uses EFSUMB levels — Level I ⩾300 supervised examinations; Level II ⩾2000 gynaecological examinations and ⩾500/year to maintain; Level III expert. O-RADS 3–5 and all inconclusive lesions are reviewed by a Level III examiner (MFM/gynaecological sonologist) before the report is finalised.
- **Students and trainees (WSU placements):** acquire images and complete the IOTA worksheet; the supervising sonographer and reporting specialist verify measurements and ADNEX inputs.
- **Worksheet:** an electronic IOTA/O-RADS worksheet in the reporting system so mandatory fields (menopausal status, solid component size, papillary count, colour score, ascites) cannot be left blank.
- **Audit (annual):** proportion of reports with complete IOTA fields and an O-RADS category (target ⩾95%); BD/ADNEX/O-RADS category vs histology or ⩾12-month follow-up outcome; malignancy rate per O-RADS category vs expected bands; time from O-RADS 4–5 report to referrer phone call (target same day); surgery rate for O-RADS 2 lesions.
- **Review:** protocol owner reviews annually and when ACR, IOTA, ASUM or RANZCOG publish new guidance; multidisciplinary case review with gyn-oncology for discordant cases.

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